

--SUMMARY--

Decision No. 464/11

01-Nov-2013

M.Crystal - B.Young - F.Jackson

- Arising out of employment (added peril)
- Seizures

The worker appealed a decision of the Appeals Resolution Officer denying entitlement for injuries suffered in falls at work in May 2005 and October 2005.

The Panel accepted the opinion of a Tribunal medical assessor that the falls were caused by seizures. The seizures were not related to work but, rather, were likely related to excessive alcohol.

The Panel also found that there was no added peril in the work environment that would have contributed to the severity of the injuries suffered in the falls.

The appeal was dismissed.

11 Pages

References: Act Citation

- WSIA

Other Case Reference

- [w5013s]
- CROSS-REFERENCE: Decision No. 464/11E
- TRIBUNAL DECISIONS CONSIDERED: 2538/11 consd

Style of Cause:

Neutral Citation: 2013 ONWSIAT 2354



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 464/11

BEFORE:

M. Crystal : Vice-Chair
B. M. Young : Member Representative of Employers
F. Jackson : Member Representative of Workers

HEARING:

December 11, 2012 at Windsor
Oral
Post-hearing activity completed on July 11, 2013

DATE OF DECISION:

November 1, 2013

NEUTRAL CITATION:

2013 ONWSIAT 2354

DECISION UNDER APPEAL:

WSIB ARO decision dated August 20, 2009

APPEARANCES:

For the worker:

Ms. S. Dajczak, Lawyer

For the employer:

Did not participate

Interpreter:

Ms. G. Gombai, Hungarian

REASONS

(i) Introduction

- [1] This appeal was heard, in Windsor, on December 11, 2012. The worker appeals the decision of Appeals Resolution Officer (ARO) L. Lum, dated August 20, 2009. That decision determined that the worker does not have initial entitlement to benefits for an accident that occurred on May 15, 2005 or entitlement for a seizure that occurred on October 8, 2005.
- [2] The worker appeared and was represented by Ms. Suzanne Dajczak, lawyer. The employer did not participate in the appeal. The worker testified at the appeal hearing. At the hearing, submissions on the factual aspect of the appeal were provided by Ms. Dajczak.
- [3] As is noted below, the Panel sought further medical information in relation to this appeal from a Tribunal Medical Assessor. Following the delivery of a report by the Assessor, the Tribunal invited Ms. Dajczak to provide written submissions on behalf of the worker, however, in correspondence, dated July 10, 2013, Ms. Dajczak advised the Tribunal that she had “no further submissions in this matter.”

(ii) The issue on appeal

- [4] The issue to be determined in this appeal is whether the worker has initial entitlement to benefits an accident that occurred on May 15, 2005 or entitlement for a seizure that occurred on October 8, 2005.

(iii) Findings of fact made in this appeal

- [5] At the hearing of this appeal, the Panel determined that it wished to put certain questions to a Tribunal Medical Assessor before rendering its final decision in this appeal. With the assistance of the Tribunal’s Medical Liaison Office (MLO), the Panel determined that it wished to obtain an expert medical opinion from a Medical Assessor in relation to the worker’s appeal. So that the Medical Assessor would have a factual basis upon which he could answer the questions posed, we made certain findings of fact concerning the worker’s accident and medical history, which were set out in a memo, dated December 11, 2012, to Tribunal Counsel Office. At the conclusion of the memo, the Panel provided its findings of fact. The memo stated:

The Panel in this appeal completed hearing evidence in this appeal today in Windsor. We seek the assistance of TCO to retain a Tribunal Medical Assessor to provide additional medical information to assist the Panel with this appeal.

The Panel is not planning to release an interim decision in this appeal, and the panel’s findings of fact are set out below in this memorandum.

In the circumstances of this appeal, the worker was a painter/decorator, employed by the accident employer, a painting company, when on May 17, 2005, he fell and sustained a laceration to his forehead and sustained a hematoma and swelling at the frontal bone of the skull. The worker was taken to hospital by ambulance, and the case materials included an Ambulance Call Report, dated May 17, 2005, which stated, in part:

Chief Complaint

Possible seizure

Incident History

[Patient] was working when he had a full body seizure after falling. A witness stated [patient] was on the floor [with] full body seizure. [Patient] hit his forehead. [One inch] laceration [with] active bleed. Upon arrival patient denies any chest or neck pain nor remembered.

Another document prepared while the worker was at hospital on the date of accident, was an "Emergency Triage Report". The document stated "had witnessed seizure lasting 5 [minutes]", although it is not clear from the document who witnessed the seizure. That document also stated that the worker had no recollection of the incident.

The case materials also included a report, dated May 24, 2005, prepared by Dr. Jacob Danial, neurologist, which stated in part:

This patient was referred from [hospital] emergency. He apparently had a seizure at work a week ago. He has no recollection of the incident. He was taken by ambulance to [hospital] emergency. He had a normal CT scan in the emergency. He was given Dilantin intravenously and was given a prescription for Dilantin.

No previous history for seizures. He is a healthy person otherwise. He does not smoke. He drinks excessively. He has done that for a while. He says he was not drinking for two or three days prior to the seizure.

No other medical problems. He works as a painter for industrial buildings.

On examination patient is alert and oriented. Blood pressure is 100/70. He has normal heart sounds. Fundus is normal and no field defects. Extremities reveal no deficits.

Patient likely had a grand mal seizure. CT scan was normal and I am wondering if overuse of alcohol may be the reason for it. It was strongly suggested that he not drink alcohol again. He should continue taking the Dilantin.

At the hearing the worker testified that at the time of the incident on May 17, 2005, he was painting a cafeteria in a building at a university. He stated that he could recall that, at one point during the day, he moved a slotted garbage bin in the cafeteria, and he noticed a puddle of liquid at the side of the bin, which was about eight inches in diameter. He stated that he slipped on that puddle of liquid, and that he fell face first into steel mesh cafeteria chairs which were bolted to the floor, lacerating his forehead. He stated that initially, he could not recall any details about the incident that occurred on May 17, 2005, but that subsequently, perhaps two weeks after the incident, he was able to recall that he slipped on this puddle of liquid.

The worker testified that the employer told him to take one week off work after the incident on May 17, 2005, and that he returned to work about a week after the incident. He testified that after returning to work, he did not lose any time from work until October 8, 2005.

At the hearing, the worker testified that on October 8, 2005, he sustained injuries as a result of another incident that occurred on that date, while he was working at a high school. The case materials included a Worker's Report of Injury (Form 6), which stated, in part:

Describe what happened to cause your injury.

I started to paint a piece of wall 10' high and 20' wide. A 3 section scaffold was in my way too close to the wall which I wanted to paint. I tried to push the scaffold away from this wall. My feet slipped on something and I fell face down in debris, I twisted my ankle and my forehead was bleeding. I get up meanwhile nobody was around me, I went outside to see my workmates for help. My boss, owner of the company called an ambulance. I don't remember losing consciousness. I was checked out at the hospital and sent home, 10 minutes

after I arrived at home I lost consciousness and was brought back to the hospital once again.

At the hearing, the worker provided a history of the incident that occurred on October 8, 2005, that was similar to that which is set out in the worker's Form 6. He stated that he fell face first after tripping on an electrical wire, and that he sustained a laceration on his face, an injury to the neck, back, his ankle and to his teeth, as well as other injuries. He testified that at the October 8, 2005 incident, he did not have a seizure at the time that he fell. He stated that he remained conscious and aware until he came outside to see his co-workers at the parking lot, but that he had a seizure at about the time that he met his co-workers outside after he fell.

The worker was seen on October 11, 2005, by Dr. A Mustafa, neurologist. Dr. Mustafa's report of that date stated that the worker was seen at hospital on October 8, 2005, but discharged home. The report stated that the worker's wife observed a further seizure at home, shortly after he arrived home.

Dr. Mustafa's report stated that the worker "is a 50 year old gentleman with epilepsy", that "he is in denial" and that "he denies any history of seizures." The report indicated that worker had not been compliant with his treatment.

At the hearing, the worker testified that he did not recall whether he had been drinking alcohol between the incident in May 2005 and October 2005. He stated that, during that period, he did not always take the medication that had been prescribed for him, according to his doctor's direction. He stated that he did not return to work after the incident in October 2005 due to his injuries, such as the injuries to his neck and ankle, in keeping with his doctor's instructions. The worker testified that he has not returned to work since the incident in October 2005.

The issue under appeal

At the hearing, the panel confirmed the following as the issue under appeal in this proceeding:

Whether the worker has initial entitlement for an accident that occurred on May 17, 2005, and/or entitlement for a seizure that occurred on October 8, 2005.

Findings of fact

As noted above, the panel wishes to retain a Tribunal Medical Assessor to obtain additional medical information to assist the panel. In the ordinary case, when an assessor is retained, the panel will make findings of fact in relation to the factual basis of the case, to which the assessor must adhere, and the assessor is obliged to provide his or her medical opinion, presuming that the facts of the case occurred in the manner determined by the Panel.

In this case, however, a factual issue arises as to whether, at the time of the incident on May 17, 2005, the worker initially had a seizure which caused him subsequently to fall and sustain a laceration to his forehead, or alternatively, whether the worker slipped and fell due to factors entirely independent of a seizure, and whether he had a seizure subsequently, which may have been related to a traumatic brain injury arising from his fall. Although this question is a central factual issue in this appeal which the panel will need to address in order to dispose of the appeal, it is also a question upon which the panel is seeking to have a better understanding after receiving the report from the assessor, in that the medical information provided by the assessor may shed light on this question. Accordingly, the panel will refrain from making a factual determination on this central question until it receives the report from the assessor.

Notwithstanding this limitation, we make the following findings of fact:

1. The worker sustained a laceration to his forehead and hematoma to the frontal skull bone after making contact with a metal chair, or another hard object, on May 17, 2005.
2. The worker had been subject to excessive alcohol consumption during the period prior to the incident on May 17, 2005, although the medical information does not indicate that the worker was intoxicated at the time of the incident, and we find that he was not intoxicated at the time of the incident.
3. The worker was taken to hospital following the incident on May 17, 2005, he was diagnosed with epilepsy and he was prescribed the drug, Dilantin, to treat the epilepsy.
4. The worker took one week off work after the incident on May 17, 2005, returning to work on or about May 24, 2005. The worker continued at work after that date without incident until October 8, 2005.
5. Between May 17, 2005 and October 8, 2005, the worker was not compliant with the treatment prescribed for him, in that he did not take the Dilantin prescribed for him in the manner prescribed by his physician.
6. The worker fell at work again on October 8, 2005, sustaining lacerations and other injuries. He was taken to hospital following this incident, and he was subsequently discharged home. Shortly after returning home, his wife observed the worker having a further seizure.
7. Until it receives further medical information from the Tribunal Medical Assessor, the panel defers making a finding about whether the worker's falls on May 17, 2005 and October 8, 2005, respectively, were preceded by a seizure, or alternatively, whether the seizure, in either case, occurred subsequent to a fall.

Questions for the Tribunal Medical Assessor

The panel wishes to pose to the Medical Assessor the questions that are proposed in the Hearing Ready Letter, dated April 13, 2012. In addition, the panel also wishes to pose this additional question to the Medical Assessor:

Based on your understanding of the worker's condition as described in the case materials, please provide the Panel with your view on whether it is more probable that the seizures were related to a traumatic brain injury caused by a fall (i.e., that the seizures were sequelae of the falls), or whether it is more probable that the worker's falls and related injuries occurred subsequent to his seizures (i.e., that the falls were sequelae of seizures).

Please obtain the necessary consent from the worker and take any other steps to retain the assessor in this appeal. Please contact me should you require any further clarification or instructions.

(iv) Medical opinion from the Tribunal Medical Assessor, Dr. Andrew Kertesz, neurologist

- [6] As noted above, in order to assist us in addressing the issues in this appeal, the Panel sought the assistance of a Tribunal Medical Assessor. A medical report, dated March 7, 2013, was requested and obtained from Dr. Andrew Kertesz, neurologist, who was retained as the Tribunal Medical Assessor for this appeal. The report provided answers to the questions posed by the Panel in relation to the appeal, which were set out in correspondence, dated February 26, 2013, from the Tribunal to Dr. Kertesz. That correspondence set out the questions, as follows:

1) An opinion based on the findings of fact as noted in the Memorandum, and a review of the case materials;

2) Succinct answers to several key questions:

1. Based on a review of the medical records in the case materials, do you agree with the worker's:
 - a. Diagnosis of seizures? Please provide your medical reasoning.
 - b. Diagnosis of epilepsy? Please provide your medical reasoning.
2. Please comment on any relationship between the workplace incidents of May 17, 2005 and/or October 8, 2005, as described by the Panel in their findings of fact, and the seizures experienced by the worker.
 - a. Would either workplace incident have provoked or triggered the seizure activity?
 - b. Is this medically likely? Please explain your reasoning.
3. Based on your understanding of the worker's condition as described in the case materials, please provide the Panel with your view on whether it is more probable that the seizures were related to a traumatic brain injury caused by a fall (i.e., that the seizures were sequelae of the falls), or whether it is more probable that the worker's falls and related injuries occurred subsequent to his seizures (i.e., that the falls were sequelae of seizures).
4. Can you provide any other information, which you feel would be helpful to the Panel and parties in this appeal?

[7] Dr. Kertesz provided a report, dated March 7, 2013, addressing the issues raised by the questions posed by the Panel. The report began by reproducing most of the Panel's memo, dated December 11, 2012, which is set out above. The report went on to state:

OPINION:

It is more probable that the falls were the sequelae of seizures.

The case material includes strong medical evidence that the worker had adult onset seizures associated with falls at the work place and at home. He was seen by two neurologists who found no other cause and attributed the seizures to excessive alcohol intake. Alcohol related seizures often occur at a time when the patient is actually sober and they are related to alcohol withdrawal (the brain becomes more excitable when the sedative effect of alcohol is removed). At the time of the first fall at ground level on May 17, 2005, a seizure was witnessed and it is likely he fell because of the seizure. It is less likely that the trauma caused the seizure although post-concussive seizures can occur. However, they are usually a single phenomenon and they are not likely to return as they have in his case. At the time of the second, apparently unwitnessed fall, in October 2005, he was again witnessed to have a seizure shortly after the fall and also at home his wife also witnessed a convulsion. He admitted to not taking his medication after his first seizure, had several more seizures in 2006 and in 2007 and had subtherapeutic levels of Dilantin.

Investigation was negative for other etiologies and the CT scan after the first episode showed only the scalp hematoma. After another fall, at home on November 30, 2007, he had increasing headaches and was noted to have a frontal subdural hematoma (not to be confused with the frontal scalp hematoma, aka. a goose egg, at the time of the first 2005 fall) on the December 10, 2007 CT scan of the brain (a new finding, related to this fall at home) and was seen by Dr. Morasutti, a neurosurgeon, but did not have surgery. A repeat CT on December 31, 2007, showed a resolution of the hematoma

Answers to Questions from the Hearing Ready Letter, April 13, 2012:

1. I agree with the diagnosis of seizures (same as epilepsy), probably alcohol related. The worker did not have a history of young onset “idiopathic epilepsy”, or epilepsy due to a brain lesion; nevertheless, it is epilepsy.
2. Both workplace incidents were falls due to seizures.
 - a) The incidents did not likely trigger epilepsy. The first one was clearly observed to be an epileptic attack and the second one was less clearly described, but was followed by two seizures and represents a series of seizures typical of severe, in this case alcohol related, epilepsy.
 - b) Epilepsy immediately after severe, usually high velocity concussion is possible, but in his case it is unlikely, as the first fall was low velocity and there was no penetrating injury or intracranial hemorrhage and it was very likely due to the seizure itself. An immediate post-concussion seizure, without structural damage is usually a rare, single, rather benign event, not followed by epilepsy or a series of seizures.
3. It is more probable that the falls and related injuries occurred subsequent to the seizures.
4. Additional information: Concussive convulsions (impact seizures) are rare and benign and the evidence is strong that this is not what the worker had (see enclosed articles). Alcohol related seizures are common and the literature on this issue is abundant (last two articles). The neurologists in this case correctly made this latter diagnosis.

(v) Applicable law

- [8] The workplace accidents, as alleged, occurred, respectively, on May 17, 2005 and October 8, 2005. Accordingly, the worker’s entitlement to benefits in this appeal is governed by the *Workplace Safety and Insurance Act, 1997*.

(vi) Analysis

- [9] In the circumstances of this appeal, the worker, who was employed by the accident employer as a painter, fell at work and sustained injuries on May 17, 2005. He had another fall at work on October 8, 2005, and again sustained injuries. Both of these incidents were associated with epileptic seizures, as subsequently diagnosed. As noted above, a central question to be determined in this appeal is whether the seizures that the worker experienced on those dates occurred spontaneously, with the fall and injuries associated with the fall, being caused by the seizures, or whether, alternatively, the worker fell as a result of factors that were present in the workplace, such as a wet floor that may have caused the worker to slip and fall at the time of the May 2005 incident, or an electrical wire, that may have caused the worker to trip and fall at the time of the October 2005 incident.
- [10] This central question was the subject of consideration by Dr. Kertesz, the Tribunal Medical Assessor in this appeal. In response, Dr. Kertesz stated in his report, dated February 26, 2013, that “both workplace incidents were falls due to seizures.” Although the report provided by Dr. Kertesz is set out above, we note that Dr. Kertesz’s reasons for this conclusion were:
- The worker was seen by two neurologists who “found no other cause and attributed the seizures to excessive alcohol intake” noting that “alcohol related seizures often occur at a time when the patient is actually sober and they are related to alcohol withdrawal”;

- Although it is possible for seizures to be caused post-concussively, in such circumstances “they are usually a single phenomenon and they are not likely to return as they have in his case”;
- In relation to the second seizure, the worker “admitted to not taking his medication after the first seizure”;
- The seizures were “probably alcohol related”;
- The first seizure was “clearly observed to be an epileptic attack”, both seizures were followed by further seizures, and “an immediate post-concussion seizure, without structural damage is usually a rare, single, rather benign event, not followed by epilepsy or a series of seizures”;
- “Concussive convulsions (impact seizures) are rare and benign and the evidence is strong that this is not what the worker had...”;
- “...the first fall was low velocity and there was no penetrating injury or intracranial hemorrhage and it was very likely due to the seizure itself”;
- “Alcohol related seizures are common and the literature on this issue is abundant...”; and
- “The neurologists in this case correctly made [the diagnosis of alcohol related seizures].”

[11] We agree with the views expressed by Dr. Kertesz in his report, for the reasons he provided, as noted. Although the worker testified at the hearing that the seizure in July 2005 occurred when he slipped on a wet floor, we note that the Emergency Triage Record, dated May 17, 2005, stated that the worker had “[no] recollection of incident”. Given that the worker reported that he did not have any recollection of the incident on May 17, 2005, immediately after it occurred, we conclude that his account of the incident given at the hearing held about seven and a half years later, at the hearing on December 11, 2012, to be unreliable.

[12] As for the second seizure that occurred at work on October 8, 2005, we note that it was not witnessed by anyone, and that the account that the worker provided at the appeal hearing, i.e., that he fell when he tripped on an electrical wire, was not provided by him when he received initial treatment on October 8, 2005. To the contrary, the Ambulance Call Report and the Emergency Triage Record, both dated October 8, 2005, stated respectively that “[patient] denies falling” and “[patient] doesn’t recall falling.” In light of this evidence, we again conclude that the worker’s account of the incident that occurred on October 8, 2005, is unreliable.

[13] We also have taken into account the report prepared by Dr. Jacob Danial, neurologist, dated May 24, 2005, which stated that the worker “drinks excessively”, that “he has done that for a while”, and that Dr. Danial suspected that the worker’s seizures were alcohol related. We find that Dr. Danial’s report supports the conclusion proved by Dr. Kertesz, that the worker’s epileptic seizure were related to his excessive consumption of alcohol, in or about 2005.

[14] On the basis of the medical information available to us, and in particular, the report provided by Dr. Kertesz, the Tribunal Medical Assessor, we conclude that the worker’s seizures in May and October 2005 were not caused by factors associated with work, and in particular, that the fall in May 2005 was not caused by the worker slipping on liquid, and that the fall in

October 2005, was not caused by the worker tripping on an electrical wire. Rather, we find, on a balance of probabilities, that the incidents occurred spontaneously, and were not related to workplace factors. We find that the worker's falls were due to his seizures. Accordingly, we find that the worker's seizures did not arise out of employment.

[15] Near the start of the hearing, the Panel explored with Ms. Dajczak, the manner in which the issues in the appeal ought to be properly articulated. During the discussion, the Panel asked Ms. Dajczak if she would agree that if the worker's seizures in May and October 2005 occurred spontaneously, that the appeal must be denied. Ms. Dajczak did not fully agree with this proposition, however, and referred briefly to Tribunal jurisprudence which she indicated stands for the proposition that, if a worker falls at work as a result of a non-compensable event (such as spontaneous fainting), that is not precipitated by workplace factors, entitlement may nevertheless be in order in some circumstances.

[16] The Panel asked Ms. Dajczak not to make full submissions on the point during this preliminary discussion, on the expectation that she would make submissions on the point when she made her final submissions. As noted above, however, when invited to make submissions on the merits of the appeal, after the Assessor's report had been received, she indicated that she had no submissions to make. We also note that her indication that she did not have further submissions, occurred after the panel allowed a lengthy extension to the time for her to make submissions, to allow her to consult with the worker's physician, concerning the Assessor's report.

[17] We have, however, taken into account her brief submission, made at the start of the hearing, that where a worker falls spontaneously, due to a seizure or fainting, some Tribunal decisions have determined that entitlement for injuries that occurred as a result of the fall may be in order. In this regard, we have considered *Decision No. 2538/11*, which explored the issue of "added peril", in circumstances where the worker has fainted spontaneously, but then sustains an injury as a result of the fall. That decision surveyed other Tribunal decisions related to this issue, and concluded that where a worker has a fall or faints at work, in circumstances which would otherwise be non-compensable, in order for entitlement to be in order for a subsequent injury, it is necessary to show that there was "added peril" in the workplace environment, which would make injury more likely to occur. The decision denied entitlement, on the basis that there was no "added peril" in the workplace, in that case. In this regard, the decision stated at paragraph 68:

The Panel has been asked to conclude in this appeal that there was an added peril in the workplace. In order to make such a finding, in our view, the Panel must be satisfied that there is a peril in the workplace that is in addition to the regular level of peril present in everyday life. The example of added peril described in *Decision No. 781/93* was of a worker who has a heart attack while driving heavy machinery. In this example we can see that it is the added aspect of a crash of heavy machinery that adds peril to the worker who has had a non-compensable heart attack. Since in this example the worker is driving heavy machinery in the course of his employment, the injuries sustained as a result of the crash would be considered compensable under the Act. In the instant appeal, however, the evidence does not demonstrate an aspect of added peril. The worker in the instant appeal suffered a non-compensable seizure, and fell to the ground. The ground surface in the workplace was hard, but not in any particularly unusual way that could be considered to be specific to the workplace, in the Panel's view. The Panel finds there is a lack of evidence to indicate that concrete floor surfaces are unusual in the normal course, or that it is likely that had the worker's seizure occurred elsewhere it likely would have been on a more forgiving floor surface. We conclude that there was no added peril in the workplace. We find that the worker's seizure and associated loss of consciousness led to

his fall to the floor and his inability to break his own fall. We find that the evidence indicates that the worker therefore sustained a head injury. We find that the head injury was due to the non-compensable seizure, and as there was no added peril in the workplace, we find that the head injury was also a non-compensable injury.

[18] In this case, as we have noted, there was very little evidence concerning the circumstances of the worker's falls, and we did not consider the worker's testimony relating to these circumstances to be reliable. In any event, from the available evidence, we are not able to conclude that there was any "added peril" from the environment where the worker was working on the two relevant occasions. In both circumstances, even according to the worker's account, he was not operating heavy machinery and he did not fall from a height. There may have been furniture or other items present, with which the worker may have made contact, however, we find that these items did not "add peril" to the environment, beyond that which could be expected in everyday life.

[19] For these reasons, the worker's appeal is denied.

DISPOSITION

[20] The appeal is denied.

[21] The worker does not have initial entitlement to benefits an accident that occurred on May 15, 2005 or entitlement for a seizure that occurred on October 8, 2005.

DATED: November 1, 2013

SIGNED: M. Crystal, B. M. Young, F. Jackson